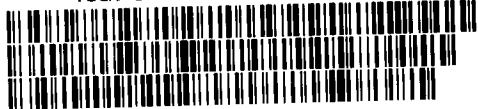


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PATHOLOGY REPORT



1CD-0-3

carcinoma, urothelial, NOS 8/20/3

Site: bladder wall, NOS 6/7/9

DIAGNOSIS

(A) LEFT DISTAL URETER. INK AT MARGIN:

Segment of ureter, no tumor present.

(B) RIGHT DISTAL URETER, INK AT MARGIN:

Segment of ureter, no tumor present.

(C) BLADDER, PROSTATE AND SEMINAL VESICLES:

BLADDER -

UROTHELIAL CARCINOMA NON-PAPILLARY TYPE, HIGH GRADE WITH FOCAL SQUAMOUS DIFFERENTIATION.

TUMOR INVADES BLADDER WALL TRANSMURALLY AND EXTENDS TO PERIVESICAL FAT.

Serosal surface, free of tumor.

No lymphovascular invasion is identified.

Resection margins, free of tumor.

PROSTATE -

UROTHELIAL CARCINOMA EXTENDS TO THE PERIURETHRAL PROSTATIC DUCTS AND SHOWS SUPERFICIAL INVASION OF THE PERIDUCTAL STROMA.

Prostatic tissue with glandular atrophy, paribasal cell hyperplasia with atypia consistent with treatment effect.

No evidence of residual prostatic carcinoma.

Prostatic tissue with chronic and focally acute inflammation.

(D) LEFT EXTERNAL ILIAC LYMPH NODES:

Four lymph nodes, no tumor present (0/4).

(E) LEFT OBTURATOR LYMPH NODES:

Nine lymph nodes, no tumor present (0/9).

(F) LEFT COMMON ILIAC LYMPH NODES:

Four lymph nodes, no tumor present (0/4).

(G) RIGHT OBTURATOR LYMPH NODES:

Eight lymph nodes, no tumor present (0/8).

(H) RIGHT EXTERNAL ILIAC LYMPH NODES:

Four lymph nodes, no tumor present (0/4).

(I) RIGHT COMMON ILIAC LYMPH NODES:

One lymph node, no tumor present (0/1).

(J) LEFT URETER, FINAL MARGIN:

Wall of ureter with unremarkable urothelium.

(K) RIGHT URETER, FINAL MARGIN:

Wall of ureter with unremarkable urothelium.

lw
6/1/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HPA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Tumor		
Case is (circle): QUALIFIED / DISQUALIFIED		
Reviewer Initials	lw	6/1/11

GROSS DESCRIPTION

(A) LEFT DISTAL URETER – A segment of ureter (0.8 cm in length) with marked true margin. The margin submitted en face for frozen in A.

*FS/DX: URETER, NEGATIVE FOR CARCINOMA OR HIGH-GRADE DYSPLASIA. BAC:

(B) RIGHT DISTAL URETER – A 1.5 cm segment of ureter with true margin marked. The margin submitted en face for frozen section in B.

*FS/DX: URETER, NEGATIVE FOR CARCINOMA OR HIGH-GRADE DYSPLASIA. BAC:

(C) BLADDER, PROSTATE AND SEMINAL VESICLES – A radical cystoprostatectomy specimen consisting of a urinary bladder (12 x 10 x 2 cm opened), prostate gland (4.5 x 4.5 x 3 cm), seminal vesicles and attached perivesical adipose tissue measuring overall 16 x 14 x 4 cm. Located in the left lateral wall and anterior wall of the bladder is an exophytic tumor (5.5 x 3.9 cm). The tumor invades into the bladder wall and extends into the perivesical fat. The tumor does not extend to the inked margin of the specimen. The right and left ureteral orifices are not involved by tumor.

The prostatic tissue is firm. No gross lesions are identified within the prostate. Tissue is submitted for tumor bank. Representative section of bladder wall and attached prostate are submitted as per enclosed map of cystoprostatectomy specimen.

SECTION CODE: C1-C47, sections of the bladder including tumor submitted sequentially from the dome to the bladder base; C48-C57, sections of the prostate gland including seminal vesicles; C58, urethral margin.

(D) LEFT EXTERNAL ILIAC LYMPH NODE – A 4.0 x 3.0 x 1.2 cm aggregate of fibroadipose tissue. The tissue is dissected to reveal five possible lymph nodes ranging from 0.5 x 0.4 x 0.3 cm-2.0 x 0.6 x 0.5 cm.

SECTION CODE: D1, two possible lymph nodes; D2, two possible lymph nodes; D3, one possible lymph node serially sectioned.

(E) LEFT OBTURATOR LYMPH NODE – A 6.0 x 3.0 x 2.0 cm aggregate of fibroadipose tissue. The tissue is dissected to reveal ten possible lymph nodes ranging from 0.6 x 0.5 x 0.3 cm-2.2 x 1.7 x 0.7 cm.

SECTION CODE: E1, two possible lymph nodes; E2, three possible lymph nodes; E3, one possible lymph node serially sectioned; E4, one possible lymph node serially sectioned; E5, one possible lymph nodes serially sectioned; E6, one possible lymph nodes serially sectioned; E7, E8, one possible lymph node serially sectioned.

(F) LEFT COMMON ILIAC LYMPH NODES – A 3.0 x 2.2 x 1.0 cm aggregate of fibroadipose tissue. The tissue is dissected to reveal four possible lymph nodes ranging from 0.6 x 0.3 x 0.3 cm-2.0 x 1.0 x 0.5 cm.

SECTION CODE: F1, three possible lymph nodes; F2, F3, one possible lymph node serially sectioned. Entire specimen is submitted.

(G) RIGHT OBTURATOR LYMPH NODE – A 5.0 x 5.0 x 1.5 cm aggregate of fibroadipose tissue. The tissue is dissected to reveal eight possible lymph nodes ranging from 0.2 x 0.2 x 0.2 cm-3.5 x 0.8 x 0.5 cm.

SECTION CODE: G1, one possible lymph node; G2, two possible lymph nodes; G3, two possible lymph nodes; G4, one possible lymph node serially sectioned; G5, one possible lymph node serially sectioned; G6, G7, one possible lymph node serially sectioned.

(H) RIGHT EXTERNAL ILIAC LYMPH NODE – A 4.7 x 4.0 x 1.8 cm aggregate of fibroadipose tissue. The tissue was dissected to reveal five possible lymph nodes ranging from 0.5 x 0.3 x 0.2 cm-2.5 x 1.7 x 0.8 cm.

SECTION CODE: H1, one possible lymph node; H2, one possible lymph node serially sectioned; H3, one possible node serially sectioned; H4, H5, one possible lymph node serially sectioned; H6, H7, one possible lymph node serially sectioned.

(I) RIGHT COMMON ILIAC LYMPH NODE – A 2.0 x 2.0 x 1.3 cm lymph node.

SECTION CODE: I1 and I2, one possible lymph node serially sectioned. Entire specimen is submitted.

(J) LEFT URETER, FINAL MARGIN – A 1.8 x 0.5 cm portion of ureter that is oriented by the surgeon with a suture. The side where the suture is inked blue and the opposite side is inked black.

SECTION CODE: J1, side with the suture, en face; J2, side opposite suture, en face.

(K) RIGHT URETER, FINAL MARGIN – A 2.0 x 0.2 cm portion of ureter that is oriented by the surgeon with a suture on one end. The side where the suture is inked blue and the opposite side is inked black.

SECTION CODE: K1, side with suture, en face; K2, side opposite suture, en face.

CLINICAL HISTORY

None given.